

METRO HEALTH CITY HOSPITAL - INTENSIVE CARE & SPECIALTY UNITS

PATIENT CASE FILE: MH-2026-884

VALIDATION DATA SHEET FOR COMPLIANCE TESTING

[DOCUMENT 1: EMERGENCY ROOM ADMISSION RECORD & CLINICAL WRAP]

DATE OF ADMISSION: 2026-02-26 (Admitted at 21:14)

ADMITTING UNIT: Emergency Department / Step-Down ICU (SDICU)

CHIEF COMPLAINT:

64-year-old female presents with multiple episodes of watery diarrhea, severe persistent vomiting, high fever, and altered cognitive sensorium for 48 hours.

INITIAL CLINICAL ASSESSMENT & ER IMPRESSION:

1. Acute Gastroenteritis with Critical Dehydration / Signs of Shock
2. Complicated Urinary Tract Infection (UTI) vs. Acute Pyelonephritis
3. Severe Diabetic Ketoacidosis (DKA) secondary to uncontrolled Metabolic Stress

PAST HISTORY / CLINICAL MATRIX:

- Diagnosed Type 2 Diabetes Mellitus (T2DM) x 8 years (reported non-compliance with standard oral agents; historical reliance on alternative therapies).
- Secondary Hypothyroidism currently on thyroid replacement therapy.
- Chronic osteoarthritis and transient history of hip region joint pain.

ACTIVE INPATIENT WARD/ICU TREATMENT ARCHIVE:

- Inj. Meromac (Meropenem) 1gm IV every 8 hours
- Inj. Pantodac (Pantoprazole) 40mg IV twice daily
- Inj. Emeset (Ondansetron) 4mg IV every 8 hours
- Inj. Sumol (Paracetamol) 1gm IV SOS for temperature elevations
- Inj. Lantus (Insulin Glargine) 10 Units Subcutaneous once daily at bedtime
- Inj. Human Actrapid Insulin titrated via intensive subcutaneous sliding scale protocol
- Inj. Sodium Bicarbonate 25ml administered in normal saline matrix for profound metabolic acidosis
- IV Fluid Resuscitation: Normal Saline (NS) and Ringer's Lactate (RL) maintained at 150 mL/hr

[DOCUMENT 2: SPECIALIST CONSULTATIONS & PROGRESS TIMELINES]

DETAILED PROGRESS TRACK LOGS (2026-02-28 to 2026-03-01):

RENEWED INTENSIVE SUMMARY: Patient experienced high fever spikes up to 103 degrees Fahrenheit along with transient hypotensive episodes. Responded well to aggressive crystalloid support and targeted antipyretic management. Foley's catheter remains in place to record hourly urinary output metrics.

ORTHOPEDIC CONSULTATION NOTE (2026-03-01): Evaluated for severe persistent hip region tenderness with restricted baseline ranges of motion. Historical tracking reveals a history of surgical intervention on the tibia 3 years prior. Summary: Suspected transient synovitis vs. osteoarthritis exacerbation. Advise plain film X-ray

tracking post-discharge.

[DOCUMENT 3: DIAGNOSTIC MATRIX, LABORATORY FLOWS & IMAGING LOGS]

BIOCHEMICAL & HEMATOLOGICAL PROFILES:

- Admission Blood Glucose: 443 mg/dL -> Capillary GRBS peaking at 496 mg/dL.
- HbA1c (External Lab Verification): 13.9% (indicating critical, chronic poor glycemic state).
- Serum Creatinine: 1.65 mg/dL on admission (consistent with AKI on presentation), trending down to 1.17 mg/dL and stabilizing at 1.04 mg/dL on 2026-03-01.
- Serum Sodium: 128 mmol/L on chemistry panel; ABG configuration mapping critical low metrics at 114 mmol/L.
- Serum Potassium: 2.65 mmol/L (Severe hypokalemia requiring corrective matrix addition).
- C-Reactive Protein (CRP): 88.23 mg/L (Highly elevated inflammatory signature).
- Widal Screening: Salmonella typhi 'O' tracking positive at 1:160 dilutions; 'H' parameter tracking positive at 1:80 dilutions.

URINALYSIS LOG MATRIX:

- Chemistry: Ketone bodies highly positive (+++), Glycosuria noted, Albuminuria present (++). Pus cells tracking at 10-12/hpf with persistent granular casts present.

MICROBIOLOGY ARCHIVE:

- Stool Matrix: Positive for red blood cells (2-3/hpf) and extensive sheets of pus cells.
- Urine Culture & Sensitivity Array: Final analysis displays no significant bacteriuria (Colony count tracking under 10,000 CFU/mL).

IMAGING RECONSTRUCTION (USG & CT KUB PLAIN):

- USG Summary: Mild hepatomegaly with Grade I fatty infiltration signatures, single cholelithiasis calculus tracking at 13mm within gallbladder neck, minimal ascites, and minimal right-sided pleural effusion.
- CT KUB plain tracking: Confirms bilateral perinephric fat plane stranding and explicit facial thickening consistent with Acute Bilateral Pyelonephritis architecture. Tiny 1-2mm left renal calyceal calculus logged.

[DOCUMENT 4: PHYSICIAN DISCHARGE DESK & PRESCRIPTION ADVICE]

DATE OF DISCHARGE SUMMARY: 2026-03-02

DISCHARGE CONDITION: Hemodynamically stable. Discharged at direct request of family attenders.

FINAL DIAGNOSTIC FACE SHEET CODE SHEET:

Principal Diagnoses: Acute Gastroenteritis with Dehydration, Urinary Tract Infection.

Secondary Diagnoses: Cholelithiasis without cholecystitis, Suspected Synovitis (hip joint).

DISCHARGE MEDICATIONS PRESCRIPTION PLAN:

1. Tab. Racer (Rabeprazole) 40mg, 1 tablet once daily before food for 7 days.
2. Tab. Emeset (Ondansetron) 4mg, 1 tablet three times daily for 3 days.
3. Tab. Oflox TZ (Ofloxacin + Tinidazole), 1 tablet twice daily for 5 days.
4. Tab M Strong, 1 tablet once daily for 15 days.
5. Tab. Zedott (Racecadotril), 1 tablet three times daily for 3 days.
6. Tab. Entro, 1 tablet twice daily for 3 days.
7. Tab. Meftal Spas (Mefenamic Acid + Dicyclomine), 1 tablet as needed (SOS) for abdominal spasm pain limits (Dispense 4 tablets total).

8. Tab. Loperamide (Loperamide) 2mg, 1 tablet twice daily for 5 days.

FOLLOW-UP DIRECTIVES: Review immediately if fever spikes return, vomiting recurs, or generalized fatigue worsens. Recheck for baseline evaluation on 2026-03-09 with updated CBC matrix parameters. Plain X-ray of pelvis and BL hips AP view recommended per orthopedic specialty guidance. *Note: Urine culture and sensitivity reports remain pending at discharge desk.*